

#1 · Diseases in the Elderly

What is the dominant heart failure phenotype in patients over 75?

Answer

Heart failure with preserved ejection fraction (HFpEF), driven by arterial stiffening and a stiff, poorly relaxing ventricle.

#2 · Diseases in the Elderly

What ejection fraction cutoff is typically used to define HFpEF?

Answer

An ejection fraction of 50 percent or higher, combined with echocardiographic or biomarker evidence of diastolic dysfunction.

#3 · Diseases in the Elderly

What scoring systems help confirm an ambiguous HFpEF diagnosis?

Answer

The H2FPEF score and the HFA-PEFF score.

#4 · Diseases in the Elderly

Which medication class now has strong guideline evidence across the full ejection fraction spectrum, including HFpEF?

Answer

SGLT2 inhibitors, such as dapagliflozin and empagliflozin.

#5 · Diseases in the Elderly

Why is spironolactone risky in elderly HFpEF patients with reduced GFR?

Answer

It carries meaningful hyperkalemia risk given the near-universal decline in renal function with age.

#6 · Diseases in the Elderly

What is a common iatrogenic complication of an aggressive diuretic dose increase in a frail elder?

Answer

Overdiuresis causing orthostatic hypotension, falls, and acute kidney injury.

#7 · Diseases in the Elderly

How many points does age 75 or older contribute to the CHA2DS2-VASc score?

Answer

2 points.

#8 · Diseases in the Elderly

What is the purpose of the HAS-BLED score?

Answer

To identify modifiable bleeding risk factors, not to serve as a reason to withhold anticoagulation outright.

#9 · Diseases in the Elderly

How many falls per year would a patient need to sustain for anticoagulation bleeding risk to outweigh stroke-prevention benefit?

Answer

Approximately 300 falls per year, according to published risk estimates.

#10 · Diseases in the Elderly

What are the three apixaban dose-reduction criteria?

Answer

Age 80 or older, weight 60 kg or less, and creatinine 1.5 mg/dL or higher; meeting two of three triggers the reduced dose.

#11 · Diseases in the Elderly

What is the reduced apixaban dose when two of three dose-reduction criteria are met?

Answer

2.5 mg twice daily.

#12 · Diseases in the Elderly

Why are DOACs generally preferred over warfarin in eligible elderly patients?

Answer

Lower rates of intracranial hemorrhage, predictable pharmacokinetics, and no need for routine INR monitoring.

#13 · Diseases in the Elderly

What is the first-line rate control strategy for most asymptomatic older adults with atrial fibrillation?

Answer

Rate control with a beta-blocker, non-dihydropyridine calcium channel blocker, or low-dose digoxin.

#14 · Diseases in the Elderly

Why does digoxin require lower doses and closer monitoring in older adults?

Answer

Because it is renally cleared and age-related decline in renal function increases toxicity risk.

#15 · Diseases in the Elderly

What defines orthostatic hypotension?

Answer

A drop in systolic blood pressure of 20 mmHg or more, or diastolic of 10 mmHg or more, within 3 minutes of standing.

#16 · Diseases in the Elderly

What is post-prandial hypotension and when should it be screened for?

Answer

A drop in blood pressure within 2 hours of eating from splanchnic vasodilation; screen with pre- and post-meal blood pressure checks.

#17 · Diseases in the Elderly

What condition should be suspected in older men with syncope during head-turning or tight collar use?

Answer

Carotid sinus hypersensitivity, evaluated with monitored carotid sinus massage.

#18 · Diseases in the Elderly

What age-related change explains blunted compensatory tachycardia during a blood pressure drop?

Answer

Reduced baroreceptor sensitivity.

#19 · Diseases in the Elderly

What is the single most effective intervention for polypharmacy-driven orthostatic hypotension?

Answer

Deprescribing the medications contributing the greatest hypotensive burden.

#20 · Diseases in the Elderly

What key counseling point is essential when starting midodrine?

Answer

The patient must remain upright for several hours after each dose to avoid supine hypertension.

#21 · Diseases in the Elderly

What three tests should always be included in an initial syncope evaluation in older adults?

Answer

Orthostatic vital signs, a medication reconciliation, and an ECG.

#22 · Diseases in the Elderly

What symptoms often replace classic chest pain in elderly patients having acute coronary syndrome?

Answer

Dyspnea, diaphoresis, syncope, delirium, or generalized weakness.

#23 · Diseases in the Elderly

What is now recognized as an independent predictor of poor outcomes after PCI, sometimes outweighing age?

Answer

Frailty status.

#24 · Diseases in the Elderly

What systolic blood pressure target did the SPRINT trial validate in ambulatory, non-frail adults over 75?

Answer

Below 120 mmHg.

#25 · Diseases in the Elderly

What blood pressure target is generally preferred for frail elders or those with limited life expectancy?

Answer

A more permissive target, often below 150/90 mmHg.

#26 · Diseases in the Elderly

Why does STOPP flag doxazosin monotherapy for hypertension in older adults?

Answer

High risk of orthostatic hypotension.

#27 · Diseases in the Elderly

What Beers Criteria interaction risks worsening renal function and blunting diuretic effect?

Answer

Combining a loop diuretic with an NSAID.

#28 · Diseases in the Elderly

Why is immediate-release nifedipine flagged as inappropriate in older adults per Beers Criteria?

Answer

Risk of hypotension and myocardial ischemia from rapid, unpredictable blood pressure drops.

#29 · Diseases in the Elderly

What percent of new heart failure diagnoses in patients over 75 are HFpEF?

Answer

Roughly half to two-thirds.

#30 · Diseases in the Elderly

What GOLD spirometric threshold defines COPD?

Answer

A post-bronchodilator FEV1/FVC below 0.70.

#31 · Diseases in the Elderly

When should inhaled corticosteroids be added to COPD therapy per GOLD algorithms?

Answer

In patients with frequent exacerbations and blood eosinophilia.

#32 · Diseases in the Elderly

What risk increases with long-term inhaled corticosteroid use in older COPD patients?

Answer

Pneumonia risk.

#33 · Diseases in the Elderly

What should be assessed before escalating COPD medication in a patient with poor control?

Answer

Inhaler technique, since arthritis, tremor, or cognitive impairment often impair proper device use.

#34 · Diseases in the Elderly

What device modification helps a patient who cannot coordinate inhaler actuation with inspiration?

Answer

Adding a spacer, or switching to a breath-activated dry powder inhaler.

#35 · Diseases in the Elderly

What anticholinergic side effect can a LAMA bronchodilator worsen in older men?

Answer

Urinary retention from benign prostatic hyperplasia.

#36 · Diseases in the Elderly

Why does pneumonia often present without fever in older adults?

Answer

Altered hypothalamic thermoregulation blunts the febrile response with aging.

#37 · Diseases in the Elderly

What atypical symptoms commonly signal pneumonia in older adults instead of cough and fever?

Answer

Delirium, functional decline, tachypnea, or anorexia.

#38 · Diseases in the Elderly

What tool is used to guide site-of-care decisions in pneumonia, and what is its limitation in frail elders?

Answer

CURB-65 or the Pneumonia Severity Index; both can underestimate severity in frail patients with low functional reserve.

#39 · Diseases in the Elderly

Why is a blanket thickened-liquid diet no longer recommended for all dysphagic elders?

Answer

It can reduce quality of life without clearly reducing aspiration pneumonia rates.

#40 · Diseases in the Elderly

What conditions increase aspiration risk in older adults through pharyngeal muscle dysfunction?

Answer

Stroke, Parkinson disease, dementia, and age-related sarcopenia of pharyngeal muscles.

#41 · Diseases in the Elderly

Have feeding tubes been shown to reduce aspiration pneumonia or improve survival in advanced dementia?

Answer

No; evidence consistently shows no benefit in either outcome.

#42 · Diseases in the Elderly

How is the age-adjusted D-dimer cutoff calculated in patients over 50?

Answer

Age multiplied by 10 (in ng/mL).

#43 · Diseases in the Elderly

Why does D-dimer rise physiologically with age?

Answer

Due to age-related and inflammatory changes independent of actual thrombus formation.

#44 · Diseases in the Elderly

What is the atypical PE presentation pattern common in older adults?

Answer

Isolated dyspnea, syncope, or delirium rather than classic pleuritic chest pain and hemoptysis.

#45 · Diseases in the Elderly

What anticoagulant class is generally preferred for treating PE in eligible older adults?

Answer

Direct oral anticoagulants.

#46 · Diseases in the Elderly

What factors should be weighed when deciding on extended anticoagulation duration after unprovoked PE?

Answer

Recurrence risk versus bleeding risk, considering the patient's remaining life expectancy.

#47 · Diseases in the Elderly

Why do older adults with OSA sometimes lack the classic obesity phenotype?

Answer

OSA prevalence and presentation patterns shift with age and can occur independent of obesity.

#48 · Diseases in the Elderly

What type of sleep apnea becomes more common in older adults with heart failure?

Answer

Central sleep apnea, including the Cheyne-Stokes breathing pattern.

#49 · Diseases in the Elderly

What is the diagnostic standard for sleep apnea regardless of age?

Answer

Polysomnography.

#50 · Diseases in the Elderly

What adherence barriers are more important to address in frail or cognitively impaired CPAP users?

Answer

Proper mask fit and addressing claustrophobia or cognitive barriers to nightly use.

#51 · Diseases in the Elderly

What mechanism links delayed circulatory time to central sleep apnea in heart failure?

Answer

Delayed circulatory time and altered chemoreceptor sensitivity to carbon dioxide create ventilatory instability.

#52 · Diseases in the Elderly

What clinical frailty tools are commonly used to assess physiologic reserve in older adults?

Answer

The Clinical Frailty Scale and the Fried frailty phenotype.

#53 · Diseases in the Elderly

What two deprescribing tools should be used together for a comprehensive medication review?

Answer

STOPP (potentially inappropriate medications) and START (potentially beneficial omissions).

#54 · Diseases in the Elderly

What is the central geriatric principle for cardiopulmonary treatment intensity?

Answer

Functional status, cognition, and goals of care, not chronological age alone, should guide treatment intensity.

#55 · Diseases in the Elderly

What combination of medications carries significant additive bradycardia risk?

Answer

A beta-blocker combined with a non-dihydropyridine calcium channel blocker such as diltiazem or verapamil.

#56 · Diseases in the Elderly

Why should medication reconciliation be performed proactively rather than only after an adverse event?

Answer

Frailty, renal function, and comorbidities evolve gradually, and accumulating polypharmacy risk can precede a fall or hospitalization.

#57 · Diseases in the Elderly

What laboratory value should be rechecked within 1-2 weeks of starting an ACE inhibitor in an older adult?

Answer

Renal function and potassium.

#58 · Diseases in the Elderly

What vaccination is specifically recommended to reduce pneumonia risk in COPD patients?

Answer

Pneumococcal vaccination.

#59 · Diseases in the Elderly

Why can a normal resting oxygen saturation be misleading in a patient with combined HFpEF and COPD?

Answer

Significant desaturation may only appear during exertion, which resting measurements alone will not detect.

#60 · Diseases in the Elderly

What is the classic ECG finding that identifies atrial fibrillation on a rhythm strip?

Answer

An irregularly irregular rhythm with absence of discrete P waves.

#61 · Diseases in the Elderly

What ECG pattern describes second-degree (Mobitz type I) AV block?

Answer

A progressively lengthening PR interval followed by a non-conducted (dropped) beat, in a repeating pattern.

#62 · Diseases in the Elderly

What is the most important initial step before concluding CPAP therapy has failed?

Answer

Review adherence data (hours of nightly use) and mask fit.

#63 · Diseases in the Elderly

What natriuretic peptide pattern helps distinguish acute cardiac decompensation from a chronically elevated HFpEF baseline?

Answer

A significant rise in NT-proBNP above the patient's own known chronic baseline value.

#64 · Diseases in the Elderly

Why should a structured medication reconciliation be performed at every care transition?

Answer

It identifies discrepancies, unintentional omissions, or duplications that commonly occur between pre-admission, in-hospital, and discharge lists.

#65 · Diseases in the Elderly

What is a key reason smoking cessation still matters in patients with established COPD?

Answer

It continues to slow lung function decline and reduce exacerbation frequency, regardless of when it is started.

#66 · Diseases in the Elderly

What is the recommended approach when multiple potential causes of syncope are identified in one patient?

Answer

Address the most readily modifiable and likely contributor first while continuing to evaluate other factors.

#67 · Diseases in the Elderly

What is the recognized mechanism behind apnea-associated bradycardia during obstructive sleep apnea events?

Answer

Vagally mediated bradycardia triggered by the apneic event itself, typically improving with effective OSA treatment.

#68 · Diseases in the Elderly

What arterial change with aging raises left ventricular afterload and promotes diastolic dysfunction?

Answer

Arterial stiffening from collagen cross-linking and elastin fragmentation.

#69 · Diseases in the Elderly

How much sinoatrial node cell loss can occur by age 75?

Answer

As much as 90 percent, predisposing to sick sinus syndrome and AV block.

#70 · Diseases in the Elderly

What is the recommended anticoagulation approach for a patient with a labile INR on warfarin?

Answer

Consider switching to a DOAC given the unpredictable INR control.

#71 · Diseases in the Elderly

What comorbidity cluster is most characteristic of HFpEF risk in older adults?

Answer

Hypertension, obesity, atrial fibrillation, chronic kidney disease, and diabetes.

#72 · Diseases in the Elderly

What should be done in a high-pretest-probability (high Wells score) patient regardless of the D-dimer result?

Answer

Proceed directly to CT pulmonary angiography regardless of the D-dimer result, since a negative D-dimer does not reliably exclude PE when pretest probability is high.

#73 · Diseases in the Elderly

What is the recognized risk of thrombolytic therapy that is elevated in older adults with PE?

Answer

Higher baseline risk of intracranial hemorrhage.

#74 · Diseases in the Elderly

What examination finding on a chest radiograph helps distinguish pneumonia from a pure COPD exacerbation?

Answer

A new focal infiltrate.

#75 · Diseases in the Elderly

What should guide the decision to treat an incidentally found subsegmental pulmonary embolism?

Answer

Whether it represents a clinically significant finding warranting anticoagulation, based on overall patient risk factors.

#76 · Diseases in the Elderly

What A1c target is appropriate for a healthy older adult with long life expectancy and few comorbidities?

Answer

Less than 7.0 to 7.5 percent, similar to the target used in younger adults.

#77 · Diseases in the Elderly

What A1c target applies to an older adult with complex or intermediate health status?

Answer

Less than 8.0 percent.

#78 · Diseases in the Elderly

What A1c target applies to a very complex or poor health elderly patient, such as one in long-term care?

Answer

Less than 8.5 percent, with the priority on avoiding symptomatic hyperglycemia and hypoglycemia rather than a numeric goal.

#79 · Diseases in the Elderly

Why is hypoglycemia particularly dangerous in older adults?

Answer

Blunted counterregulatory responses and reduced awareness increase risk of falls, fractures, arrhythmia, cognitive decline, and death.

#80 · Diseases in the Elderly

Which sulfonylurea carries a strong Beers Criteria warning in older adults, and why?

Answer

Glyburide, because its active metabolite accumulates with declining renal function and causes prolonged hypoglycemia.

#81 · Diseases in the Elderly

Why is glipizide preferred over glyburide in elderly patients?

Answer

It has a shorter half-life and hepatic rather than renal-dependent metabolite clearance.

#82 · Diseases in the Elderly

At what eGFR should metformin dose be reduced, and at what eGFR should it be discontinued?

Answer

Reduce below 45 mL/min/1.73m²; discontinue below 30 mL/min/1.73m².

#83 · Diseases in the Elderly

Why should sliding scale insulin not be used as monotherapy in frail elderly patients?

Answer

It produces erratic glycemic swings without background basal coverage; scheduled basal insulin with correction dosing is preferred.

#84 · Diseases in the Elderly

What are the key safety concerns with SGLT2 inhibitors in frail elderly patients?

Answer

Volume depletion, orthostatic hypotension, falls, and genitourinary infections.

#85 · Diseases in the Elderly

What is the general benefit horizon for microvascular benefit from tight glycemic control?

Answer

About eight to ten years, which should be weighed against a patient's life expectancy.

#86 · Diseases in the Elderly

What TSH threshold is generally required before treating subclinical hypothyroidism in most adults over 65?

Answer

A TSH persistently above 10 mIU/L.

#87 · Diseases in the Elderly

What did the TRUST trial demonstrate about treating subclinical hypothyroidism with TSH below 10 mIU/L in older adults?

Answer

No symptomatic benefit from levothyroxine treatment.

#88 · Diseases in the Elderly

What is the typical starting dose and titration interval for levothyroxine in an elderly patient?

Answer

25 to 50 micrograms daily, with TSH rechecked every six to eight weeks before further titration.

#89 · Diseases in the Elderly

What are the classic clinical features of apathetic hyperthyroidism in the elderly?

Answer

Lethargy, depression, unintentional weight loss, and new atrial fibrillation, often without tremor or heat intolerance.

#90 · Diseases in the Elderly

Why might apathetic hyperthyroidism be mistaken for malignancy or depression?

Answer

It lacks the classic hyperkinetic features of thyrotoxicosis, presenting instead with weight loss and lethargy.

#91 · Diseases in the Elderly

What thyroid abnormality can amiodarone cause, and in which direction most commonly?

Answer

Amiodarone can cause either hypothyroidism or thyrotoxicosis; hypothyroidism is more common, especially with underlying autoimmune thyroid disease.

#92 · Diseases in the Elderly

What T-score defines osteoporosis on DXA?

Answer

A T-score of negative 2.5 or lower at the femoral neck, total hip, or lumbar spine.

#93 · Diseases in the Elderly

What T-score range defines osteopenia?

Answer

Negative 1.0 to negative 2.5.

#94 · Diseases in the Elderly

What clinical inputs does the FRAX tool use to estimate ten-year fracture probability?

Answer

Age, sex, BMI, prior fracture, parental hip fracture, smoking, glucocorticoid use, rheumatoid arthritis, secondary osteoporosis, and alcohol intake, with optional bone density.

#95 · Diseases in the Elderly

At what FRAX thresholds is osteoporosis treatment generally recommended?

Answer

Ten-year hip fracture risk above 3 percent or major osteoporotic fracture risk above 20 percent.

#96 · Diseases in the Elderly

Besides FRAX thresholds, what two findings independently indicate osteoporosis treatment regardless of T-score?

Answer

A prior hip or vertebral fragility fracture, or a T-score of negative 2.5 or lower.

#97 · Diseases in the Elderly

What renal function threshold is generally required for safe use of oral bisphosphonates?

Answer

An eGFR above 30 to 35 mL/min/1.73m².

#98 · Diseases in the Elderly

What is the proper administration technique for oral bisphosphonates?

Answer

Take on an empty stomach with a full glass of water and remain upright for at least 30 minutes.

#99 · Diseases in the Elderly

What are the two rare but serious complications of prolonged bisphosphonate use, generally after five or more years?

Answer

Atypical femoral fracture and osteonecrosis of the jaw.

#100 · Diseases in the Elderly

What warning sign on plain film suggests an impending atypical femoral fracture?

Answer

A transverse lucency in the lateral cortex of the femoral shaft with prodromal thigh pain.

#101 · Diseases in the Elderly

Why is denosumab useful for patients with renal impairment?

Answer

It is not renally cleared, unlike bisphosphonates, making it an option when renal function precludes their use.

#102 · Diseases in the Elderly

What is the most important risk of stopping denosumab without a transition agent?

Answer

Rebound vertebral fracture risk from a surge in bone turnover.

#103 · Diseases in the Elderly

What dosing interval must be strictly maintained with denosumab therapy?

Answer

A fixed six-month dosing interval.

#104 · Diseases in the Elderly

In which patients is anabolic osteoporosis therapy such as teriparatide generally preferred as the initial agent?

Answer

Patients at very high fracture risk, such as those with a recent fragility fracture or very low T-score.

#105 · Diseases in the Elderly

Why should anabolic osteoporosis agents generally be given before, not after, antiresorptive therapy?

Answer

An antiresorptive given first can blunt the subsequent anabolic bone-forming response.

#106 · Diseases in the Elderly

What boxed warning does romosozumab carry, and who should avoid it?

Answer

A cardiovascular event warning; it should be avoided after a recent myocardial infarction or stroke.

#107 · Diseases in the Elderly

Is there still a mandated lifetime treatment duration limit for teriparatide?

Answer

No. The FDA removed the boxed warning and the associated 2-year duration limit from the teriparatide label in 2020 after long-term human surveillance data showed no increased osteosarcoma risk, so duration is now individualized.

#108 · Diseases in the Elderly

What serum 25-hydroxyvitamin D level is generally targeted for bone health?

Answer

At least 30 ng/mL.

#109 · Diseases in the Elderly

What are typical daily targets for vitamin D and calcium supplementation in osteoporosis management?

Answer

800 to 1000 international units of vitamin D and 1000 to 1200 mg of calcium from diet plus supplementation.

#110 · Diseases in the Elderly

Why should excessive calcium supplementation be avoided?

Answer

It has been associated with cardiovascular risk signals in some studies.

#111 · Diseases in the Elderly

Approximately how much does GFR decline per year on average after age 40 with normal aging?

Answer

About 1 mL/min/1.73m² per year.

#112 · Diseases in the Elderly

Why is serum creatinine an unreliable marker of renal function in a sarcopenic elderly patient?

Answer

Reduced muscle mass lowers creatinine production, masking true renal impairment.

#113 • Diseases in the Elderly

What test can supplement creatinine for a more accurate eGFR estimate in low-muscle-mass patients?

Answer

Cystatin C.

#114 • Diseases in the Elderly

What three criteria trigger apixaban dose reduction to 2.5 mg twice daily when at least two are met?

Answer

Age 80 or older, body weight 60 kg or less, or serum creatinine 1.5 mg/dL or greater.

#115 · Diseases in the Elderly

Why does dabigatran require caution in declining renal function?

Answer

It is predominantly renally cleared and accumulates with reduced clearance, raising bleeding risk.

#116 · Diseases in the Elderly

What class of pain medication requires significant dose reduction in renal impairment and can cause sedation and confusion if not adjusted?

Answer

Gabapentin and pregabalin.

#117 · Diseases in the Elderly

What is the target digoxin level range in older adults?

Answer

0.5 to 0.9 ng/mL, lower than the target in younger patients.

#118 · Diseases in the Elderly

Why are NSAIDs relatively contraindicated in chronic kidney disease?

Answer

They cause afferent arteriolar vasoconstriction, risking acute kidney injury.

#119 · Diseases in the Elderly

What is the mechanism behind the ACE inhibitor plus diuretic plus NSAID 'triple whammy' combination?

Answer

Compounded hemodynamic stress on renal perfusion, substantially raising acute kidney injury risk.

#120 · Diseases in the Elderly

What is the most common electrolyte disorder in hospitalized and community-dwelling older adults?

Answer

Hyponatremia.

#121 · Diseases in the Elderly

What laboratory pattern defines SIADH?

Answer

Euvoemia, low serum osmolality, inappropriately concentrated urine, and urine sodium generally above 40 mEq/L without diuretic use.

#122 · Diseases in the Elderly

Name three medication classes commonly implicated in elderly SIADH.

Answer

SSRIs, carbamazepine, and thiazide diuretics.

#123 · Diseases in the Elderly

What is unusual about the timing of thiazide-induced hyponatremia?

Answer

It can occur even years after a previously stable, tolerated dose.

#124 · Diseases in the Elderly

What is the maximum safe rate of sodium correction for chronic hyponatremia over 24 hours?

Answer

No more than 8 mEq/L in 24 hours.

#125 · Diseases in the Elderly

What complication can result from overly rapid correction of chronic hyponatremia?

Answer

Osmotic demyelination syndrome.

#126 · Diseases in the Elderly

What is the first-line management for chronic SIADH-related hyponatremia?

Answer

Fluid restriction.

#127 · Diseases in the Elderly

What are second-line options for SIADH-related hyponatremia when fluid restriction fails?

Answer

Salt tablets, urea, or a vasopressin receptor antagonist.

#128 · Diseases in the Elderly

What is 'tea and toast syndrome'?

Answer

Hyponatremia worsened by very low dietary solute intake, which limits the kidney's capacity to excrete free water.

#129 · Diseases in the Elderly

How does hypovolemic hyponatremia's urine sodium differ from that of SIADH?

Answer

Hypovolemic hyponatremia typically shows a low urine sodium below 20 mEq/L, reflecting appropriate renal sodium retention, unlike SIADH.

#130 · Diseases in the Elderly

What electrolyte finding distinguishes primary adrenal insufficiency from SIADH as a cause of hyponatremia?

Answer

Adrenal insufficiency classically causes hyperkalemia alongside hyponatremia; SIADH typically does not affect potassium.

#131 · Diseases in the Elderly

What is the preferred acute gout treatment in an elderly patient with chronic kidney disease and heart failure?

Answer

Low-dose colchicine or corticosteroids, avoiding NSAIDs.

#132 · Diseases in the Elderly

What is the target serum urate level for urate-lowering therapy?

Answer

Below 6 mg/dL.

#133 · Diseases in the Elderly

How should allopurinol be started in a patient with reduced renal function?

Answer

At a low dose, generally 100 mg daily or lower, titrated slowly.

#134 · Diseases in the Elderly

What serious complication can occur with allopurinol, especially at higher starting doses in renal impairment?

Answer

Allopurinol hypersensitivity syndrome, presenting with fever, rash, and eosinophilia.

#135 · Diseases in the Elderly

Why does allopurinol interact dangerously with azathioprine?

Answer

Allopurinol inhibits xanthine oxidase, which metabolizes azathioprine, risking severe myelosuppression.

#136 · Diseases in the Elderly

Should established urate-lowering therapy be stopped during an acute gout flare?

Answer

No, it should generally be continued through the flare rather than discontinued.

#137 · Diseases in the Elderly

Why do older women on chronic diuretics more often develop atypical gout presentations?

Answer

Long-term diuretic use predisposes to atypical joint involvement, such as finger joints, and tophi formation.

#138 · Diseases in the Elderly

What is the stepwise approach to osteoarthritis management before considering oral NSAIDs?

Answer

Topical NSAIDs, acetaminophen, exercise, and weight loss.

#139 · Diseases in the Elderly

Why are opioids generally avoided as first or second line therapy for osteoarthritis in older adults?

Answer

They carry disproportionate risk of falls, delirium, constipation, and dependence relative to modest analgesic benefit.

#140 · Diseases in the Elderly

What option is reasonable for refractory single-joint osteoarthritis pain before escalating to systemic therapy?

Answer

Intra-articular corticosteroid injection.

#141 • Diseases in the Elderly

Which medication classes discussed in this chapter appear explicitly on the Beers Criteria?

Answer

Glyburide, long-acting sulfonylureas generally, and chronically used NSAIDs.

#142 • Diseases in the Elderly

What does subclinical hyperthyroidism, TSH suppressed with normal free thyroxine, increase the risk of?

Answer

Atrial fibrillation and accelerated bone loss.

#143 · Diseases in the Elderly

What can reduce levothyroxine absorption when taken concurrently, requiring separation of dosing times?

Answer

Calcium carbonate and proton pump inhibitors.

#144 · Diseases in the Elderly

What is the general risk of continuing a bisphosphonate beyond five years without reassessment?

Answer

Increased cumulative risk of atypical femoral fracture and osteonecrosis of the jaw, prompting consideration of a drug holiday.

#145 · Diseases in the Elderly

What laboratory findings should be corrected before starting a potent bisphosphonate to avoid severe post-treatment hypocalcemia?

Answer

Underlying vitamin D deficiency and hypocalcemia.

#146 · Diseases in the Elderly

How does FRAX's lack of a direct chronic kidney disease input affect its risk estimate?

Answer

FRAX may underestimate true fracture risk in patients with chronic kidney disease, which independently raises fracture risk.

#147 · Diseases in the Elderly

What does hypercalcemia with a suppressed parathyroid hormone level suggest, as opposed to an elevated PTH?

Answer

A PTH-independent cause such as malignancy, rather than primary hyperparathyroidism.

#148 · Diseases in the Elderly

Why is glycemic monitoring frequency often reduced in patients with advanced dementia and relaxed targets?

Answer

Frequent monitoring adds burden without meaningfully changing management aligned to comfort-focused goals of care.

#149 · Diseases in the Elderly

What is the guiding principle for endocrine, renal, and musculoskeletal disease management across geriatric medicine?

Answer

Individualized management based on frailty, life expectancy, renal function, and goals of care rather than rigid disease-specific numeric targets.

#150 · Diseases in the Elderly

What sick day counseling is important for patients on SGLT2 inhibitors?

Answer

Temporarily hold the medication during acute illness, dehydration, or reduced oral intake to reduce the risk of euglycemic diabetic ketoacidosis.

#151 · Diseases in the Elderly

Does age alone exclude an older adult from IV thrombolysis for stroke?

Answer

No. Age alone is not an exclusion criterion; eligible patients over 80 still derive relative benefit similar to younger patients.

#152 · Diseases in the Elderly

What is the standard IV alteplase eligibility window from last known well?

Answer

Within 4.5 hours of last known well time, based on accumulated trial evidence.

#153 · Diseases in the Elderly

How many falls per year would it take for fall risk to outweigh anticoagulation's stroke benefit?

Answer

Approximately 295 falls in one year, meaning fall risk is usually overestimated as a reason to withhold anticoagulation.

#154 · Diseases in the Elderly

What does the HAS-BLED score guide when elevated?

Answer

It should prompt correction of modifiable bleeding risk factors, not automatic discontinuation of anticoagulation.

#155 · Diseases in the Elderly

Why are DOACs generally preferred over warfarin in eligible older adults with atrial fibrillation?

Answer

They carry a lower risk of intracranial hemorrhage, though renal dosing adjustments are still required.

#156 · Diseases in the Elderly

What is the typical anticoagulation initiation delay after a large ischemic stroke?

Answer

Approximately 12 to 14 days, confirmed with repeat imaging excluding hemorrhagic transformation first.

#157 · Diseases in the Elderly

What is the typical anticoagulation initiation delay after a TIA?

Answer

Approximately 1 day, since bleeding risk from a TIA-sized event is minimal.

#158 · Diseases in the Elderly

Which score stratifies short-term stroke risk after a TIA?

Answer

The ABCD2 score, incorporating age, blood pressure, clinical features, duration, and diabetes.

#159 · Diseases in the Elderly

Within what window does carotid endarterectomy provide the greatest benefit after a symptomatic event?

Answer

Within 2 weeks of the qualifying symptomatic event, for 70 to 99 percent stenosis.

#160 · Diseases in the Elderly

What atypical stroke presentation is common in frail older adults?

Answer

Acute confusion or unexplained functional decline rather than classic focal hemiparesis.

#161 · Diseases in the Elderly

Does age alone determine eligibility for extended-window mechanical thrombectomy?

Answer

No; eligibility is guided by favorable perfusion imaging showing salvageable tissue, not chronologic age.

#162 · Diseases in the Elderly

What CHA₂DS₂-VASc contribution does age over 75 provide?

Answer

Age contributes independently to the score, so nearly all patients over 75 with atrial fibrillation score at least 2 from age alone.

#163 · Diseases in the Elderly

What is the typical anticoagulation initiation delay after a small non-disabling infarct?

Answer

Approximately 3 days, after confirming no hemorrhagic transformation on follow-up imaging.

#164 · Diseases in the Elderly

What distinguishes drug-induced parkinsonism from idiopathic Parkinson disease?

Answer

Drug-induced parkinsonism tends to be symmetric and improves after stopping the offending dopamine-blocking agent; idiopathic disease is typically asymmetric.

#165 · Diseases in the Elderly

Which medications commonly cause drug-induced parkinsonism?

Answer

Dopamine-blocking antipsychotics and antiemetics such as metoclopramide and prochlorperazine.

#166 · Diseases in the Elderly

What eye finding characterizes progressive supranuclear palsy?

Answer

Impaired downgaze, along with early postural instability and falls within the first year.

#167 · Diseases in the Elderly

What triad characterizes multiple system atrophy?

Answer

Parkinsonism, cerebellar signs, and prominent early autonomic dysfunction including orthostatic hypotension and urinary retention.

#168 · Diseases in the Elderly

How does dementia with Lewy bodies differ in timing from Parkinson disease dementia?

Answer

In Lewy body disease, cognitive symptoms arise within about a year of motor symptoms; in Parkinson disease dementia, cognition declines more than a year after motor onset.

#169 · Diseases in the Elderly

Should levodopa be delayed in older-onset Parkinson disease to avoid future dyskinesia?

Answer

No; older-onset patients have a shorter disease trajectory, so motor fluctuations and dyskinesias are less problematic even with earlier initiation.

#170 · Diseases in the Elderly

Why are dopamine agonists generally avoided as first-line therapy in older Parkinson patients?

Answer

They carry higher risk of confusion, hallucinations, and orthostatic hypotension in older patients.

#171 · Diseases in the Elderly

What nonmotor symptom often precedes the motor diagnosis of Parkinson disease by years?

Answer

Constipation, reflecting early enteric nervous system involvement.

#172 · Diseases in the Elderly

What is the initial management approach for Parkinson-related orthostatic hypotension?

Answer

Non-pharmacologic measures: increased fluid and salt intake, compression garments, and slow positional changes.

#173 · Diseases in the Elderly

What pharmacologic agents are used for Parkinson orthostatic hypotension refractory to non-pharmacologic measures?

Answer

Midodrine or droxidopa.

#174 · Diseases in the Elderly

What impulse control disorders are associated with dopamine agonist therapy?

Answer

Compulsive gambling, shopping, or hypersexuality, particularly in younger patients, and in those with a personal or family history of impulsive or addictive behaviors.

#175 · Diseases in the Elderly

Why is phenytoin dosing unpredictable in frail older patients?

Answer

Its nonlinear kinetics combined with variable albumin and renal function make free drug levels unpredictable.

#176 · Diseases in the Elderly

What is the most common identifiable cause of new-onset seizures in adults over 65?

Answer

Stroke, whether acute or as a remote sequela of prior infarct or hemorrhage.

#177 · Diseases in the Elderly

What should be suspected in an older adult with persistent altered mental status after a witnessed seizure?

Answer

Nonconvulsive status epilepticus, which requires EEG for diagnosis.

#178 · Diseases in the Elderly

Which antiepileptic is often first-line in older adults due to minimal hepatic metabolism and few interactions?

Answer

Levetiracetam, though clinicians should ask about irritability or mood changes.

#179 · Diseases in the Elderly

Why are phenytoin and carbamazepine generally avoided as first-line antiepileptics in older adults?

Answer

They interact extensively with anticoagulants and cardiovascular medications commonly used in this population.

#180 · Diseases in the Elderly

What seizure type is more common than classic tonic-clonic convulsions in older adults?

Answer

Focal seizures with impaired awareness, often with prolonged postictal confusion.

#181 · Diseases in the Elderly

Does Alzheimer disease independently increase seizure risk?

Answer

Yes, compared with age-matched controls without dementia.

#182 · Diseases in the Elderly

What titration caution applies to lamotrigine regardless of patient age?

Answer

Slow titration is required to reduce the risk of serious rash, including Stevens-Johnson syndrome.

#183 · Diseases in the Elderly

Why might treatment be started after a single unprovoked seizure in an older adult?

Answer

Recurrence risk after a first unprovoked seizure is often high enough in older adults to justify treatment even after one event.

#184 · Diseases in the Elderly

What is a key drug-interaction concern with carbamazepine in an anticoagulated older patient?

Answer

Its enzyme-inducing effect can significantly alter warfarin metabolism and anticoagulation control.

#185 · Diseases in the Elderly

What metabolic derangement is a recognized reversible cause of new-onset seizures in older adults?

Answer

Severe hyponatremia, which should be corrected alongside acute seizure management.

#186 · Diseases in the Elderly

What is the most common identifiable cause of distal symmetric peripheral neuropathy in older adults?

Answer

Diabetes mellitus, though a meaningful proportion of cases remain idiopathic even after workup.

#187 · Diseases in the Elderly

What basic laboratory panel should be obtained for new peripheral neuropathy in an older adult?

Answer

Fasting glucose or HbA1c, vitamin B12, complete blood count, comprehensive metabolic panel, and thyroid function.

#188 · Diseases in the Elderly

Why should methylmalonic acid be checked with a low-normal B12 level?

Answer

It is a more sensitive marker of tissue-level B12 deficiency than serum B12 alone.

#189 · Diseases in the Elderly

What test should be considered for atypical or rapidly progressive neuropathy?

Answer

Serum protein electrophoresis with immunofixation to evaluate for monoclonal gammopathy.

#190 · Diseases in the Elderly

What prescribing caution applies to gabapentin and pregabalin in older adults?

Answer

Dose adjustment for renal function and slow titration to avoid sedation, gait instability, and falls.

#191 · Diseases in the Elderly

What safety concern applies to duloxetine in a patient with orthostatic hypotension?

Answer

Duloxetine can worsen orthostatic hypotension and should be used cautiously in renal or hepatic impairment.

#192 · Diseases in the Elderly

Why does peripheral neuropathy independently raise fall risk in older adults?

Answer

Impaired proprioception and reduced protective sensation increase fall risk regardless of the pharmacologic regimen chosen.

#193 · Diseases in the Elderly

Why is vitamin B12 deficiency particularly relevant in older adults?

Answer

High prevalence of atrophic gastritis and other malabsorption causes increases risk of clinically significant deficiency.

#194 · Diseases in the Elderly

What clinical change should prompt reevaluation of a stable neuropathy for an atypical cause?

Answer

A shift to rapidly progressive or asymmetric neuropathy from a prior stable distal symmetric pattern.

#195 · Diseases in the Elderly

Are topical agents like lidocaine patches subject to the same renal dosing concerns as gabapentin?

Answer

No; topical agents have minimal systemic absorption and generally do not require the same renal dose adjustments.

#196 · Diseases in the Elderly

What temperature rise defines significant fever in a frail older adult?

Answer

A rise of 1.1 degrees Celsius above baseline, or a single reading above 37.8 degrees Celsius.

#197 · Diseases in the Elderly

According to IDSA guidance, when should asymptomatic bacteriuria be treated?

Answer

Only before an invasive urologic procedure likely to cause mucosal bleeding.

#198 · Diseases in the Elderly

Should a positive urine culture in a confused older adult without urinary symptoms be treated as a UTI?

Answer

No; an alternative cause of confusion should be actively sought before attributing it to bacteriuria.

#199 · Diseases in the Elderly

What tool guides pneumonia site-of-care decisions and includes age as an independent factor?

Answer

The CURB-65 score, in which age 65 or older independently contributes a point.

#200 · Diseases in the Elderly

What are the preferred first-line agents for *Clostridioides difficile* infection over metronidazole?

Answer

Vancomycin or fidaxomicin, with fidaxomicin associated with lower recurrence rates.

#201 · Diseases in the Elderly

Why might sepsis be missed in an older adult on a beta-blocker?

Answer

The expected tachycardic response to sepsis may be blunted, so a normal heart rate should not exclude sepsis.

#202 · Diseases in the Elderly

What are the three components of the quick SOFA score?

Answer

Altered mentation, respiratory rate of 22 or higher, and systolic blood pressure of 100 or lower.

#203 · Diseases in the Elderly

Within what window should antiviral therapy for herpes zoster be started?

Answer

Within 72 hours of rash onset, to reduce acute pain duration and postherpetic neuralgia incidence.

#204 · Diseases in the Elderly

Which zoster vaccine is preferred in adults 50 and older, including immunocompromised patients?

Answer

The recombinant zoster vaccine, preferred over the older live attenuated formulation.

#205 · Diseases in the Elderly

How does infective endocarditis often present in older adults?

Answer

Insidiously, with low-grade fever, malaise, weight loss, and a new or changing heart murmur.

#206 · Diseases in the Elderly

What predisposing factors for endocarditis are common in older adults?

Answer

Degenerative valve disease and prior cardiac procedures.

#207 · Diseases in the Elderly

What population has a particularly high prevalence of asymptomatic bacteriuria?

Answer

Older women and residents of long-term care facilities.

#208 · Diseases in the Elderly

What atypical infection presentation should prompt an infectious workup in older adults?

Answer

New delirium, functional decline, anorexia, or falls, even without fever or localizing symptoms.

#209 · Diseases in the Elderly

What is a common driver of inappropriate antibiotic use in geriatric practice?

Answer

Overdiagnosis and overtreatment of presumed UTI based on asymptomatic bacteriuria.

#210 · Diseases in the Elderly

What is postherpetic neuralgia?

Answer

Persistent pain in the affected dermatome after a herpes zoster rash resolves; it is more common and longer-lasting in older adults.

#211 · Diseases in the Elderly

Why are older adults more likely to develop severe or fulminant *C. difficile* infection?

Answer

Age-related physiologic changes plus higher antibiotic exposure and hospitalization rates increase severity risk.

#212 · Diseases in the Elderly

What is a recognized risk factor for *C. difficile* infection besides antibiotics and hospitalization?

Answer

Proton pump inhibitor use.

#213 · Diseases in the Elderly

What imaging should be obtained when bacteremia with a typical endocarditis organism is found?

Answer

Echocardiography, transthoracic or transesophageal, to evaluate for valvular vegetation.

#214 · Diseases in the Elderly

What genitourinary symptoms support treating a true UTI rather than asymptomatic bacteriuria?

Answer

New dysuria, urgency, frequency, or suprapubic pain.

#215 · Diseases in the Elderly

Why might pneumonia present without fever in a frail older adult?

Answer

Attenuated thermoregulation and febrile response can mask fever even with true, serious infection.

#216 · Diseases in the Elderly

At what age does the USPSTF recommend against colorectal cancer screening entirely?

Answer

Age 85; screening between 76 and 85 is an individualized decision.

#217 · Diseases in the Elderly

Through what age is mammography breast cancer screening recommended by the USPSTF?

Answer

Through age 74, with insufficient evidence to assess benefit beyond that age.

#218 · Diseases in the Elderly

When can cervical cancer screening be discontinued?

Answer

At age 65, in women with adequate prior negative screening and no history of high-grade lesions.

#219 · Diseases in the Elderly

What generally limits PSA screening recommendations past a certain age?

Answer

The long lead time between prostate cancer detection and mortality benefit relative to competing causes of death; generally not recommended past age 70.

#220 · Diseases in the Elderly

What is the purpose of a life expectancy tool such as ePrognosis in cancer screening decisions?

Answer

To estimate whether a patient is likely to live long enough to benefit from early cancer detection.

#221 · Diseases in the Elderly

Why do chronologic age and ECOG performance status alone poorly predict chemotherapy toxicity?

Answer

They fail to capture function, comorbidity, cognition, nutrition, polypharmacy, and social support that geriatric assessment tools incorporate.

#222 · Diseases in the Elderly

What outcome has geriatric assessment-guided oncology care been shown to reduce?

Answer

Grade 3 or higher chemotherapy toxicity, compared with usual care.

#223 · Diseases in the Elderly

What malignancy should always be considered with new iron deficiency anemia in an older adult?

Answer

Colorectal cancer; colonoscopy is warranted regardless of gastrointestinal symptoms.

#224 · Diseases in the Elderly

What symptom triad is classic for pancreatic cancer in older adults?

Answer

Painless jaundice, new-onset diabetes, or unexplained weight loss.

#225 · Diseases in the Elderly

What has early integration of palliative care alongside cancer treatment been shown to improve?

Answer

Quality of life, and in some studies survival, without hastening death.

#226 · Geriatric Syndromes

What proportion of community-dwelling older adults fall each year?

Answer

Approximately one in four (about 25 percent) falls at least once per year.

#227 · Geriatric Syndromes

What is the approximate one-year fall probability for an older adult with four or more identified fall risk factors?

Answer

Approximately 78 percent, compared with under 10 percent for someone with no identified risk factors.

#228 · Geriatric Syndromes

According to AGS/BGS guidance, what should trigger a comprehensive multifactorial fall risk assessment?

Answer

Reporting two or more falls in the past year, presenting for acute fall evaluation, or demonstrating abnormal gait or balance.

#229 · Geriatric Syndromes

How is the Timed Up and Go test performed?

Answer

The patient rises from a chair without using the arms if possible, walks three meters, turns, walks back, and sits down; it is timed.

#230 · Geriatric Syndromes

What Timed Up and Go time is generally considered abnormal and predictive of increased fall risk?

Answer

12 seconds or longer.

#231 · Geriatric Syndromes

What do the three letters of the CDC STEADI initiative stand for?

Answer

Screen for fall risk, Assess modifiable risk factors, and Intervene using effective interventions.

#232 · Geriatric Syndromes

Name the three core STEADI screening questions.

Answer

Have you fallen in the past year, do you feel unsteady when standing or walking, and do you worry about falling.

#233 · Geriatric Syndromes

What is the formal definition of orthostatic hypotension?

Answer

A systolic blood pressure drop of at least 20 mmHg or a diastolic drop of at least 10 mmHg within three minutes of standing.

#234 · Geriatric Syndromes

At what two time points should orthostatic blood pressure be measured after standing, and why?

Answer

At 1 and 3 minutes, since delayed orthostatic hypotension is common in older adults and can be missed with a single early reading.

#235 · Geriatric Syndromes

Why does a comprehensive fall assessment include a cardiovascular exam listening for a systolic ejection murmur?

Answer

A murmur radiating to the carotids raises concern for aortic stenosis, a cardiac cause of exertional syncope and falls.

#236 · Geriatric Syndromes

What is post-fall syndrome?

Answer

Fear of falling leading to self-imposed activity restriction, deconditioning, and social isolation after a fall.

#237 · Geriatric Syndromes

What is the recommended management approach for a patient with significant fear of falling and reduced activity?

Answer

Graded, supervised exercise and balance training combined with reassurance and gradual resumption of activity.

#238 · Geriatric Syndromes

Which structured exercise program has strong evidence for reducing fall rates in community-dwelling older adults?

Answer

The Otago Exercise Programme, which emphasizes progressive balance and strength training.

#239 · Geriatric Syndromes

In whom does vitamin D supplementation most reliably reduce fall risk?

Answer

Patients with baseline vitamin D deficiency; benefit is not uniform in vitamin D-replete populations.

#240 · Geriatric Syndromes

What is the current evidence-based role of hip protectors in fracture prevention?

Answer

They have shown inconsistent efficacy and adherence in trials and are not considered a primary prevention strategy.

#241 · Geriatric Syndromes

List the five criteria of the Fried frailty phenotype.

Answer

Unintentional weight loss, exhaustion, weakness (grip strength), slowness (gait speed), and low physical activity.

#242 · Geriatric Syndromes

What weight loss threshold defines the weight loss criterion in the Fried phenotype?

Answer

Loss of 10 pounds or more, or 5 percent or more of body weight, in the preceding year.

#243 · Geriatric Syndromes

How many Fried phenotype criteria must be met to classify a patient as frail versus prefrail?

Answer

Three or more criteria for frail; one or two criteria for prefrail; zero criteria for robust.

#244 · Geriatric Syndromes

How is the weakness criterion of the Fried phenotype measured?

Answer

Grip strength on a dynamometer, with cutoffs stratified by sex and body mass index.

#245 · Geriatric Syndromes

How is the exhaustion criterion of the Fried phenotype assessed?

Answer

By self-report on a standardized questionnaire asking if the patient felt everything was an effort or could not get going most of the past week.

#246 · Geriatric Syndromes

What large study was used to derive the original Fried frailty phenotype?

Answer

The Cardiovascular Health Study.

#247 · Geriatric Syndromes

How is the cumulative deficit frailty index calculated?

Answer

The number of health deficits present divided by the total number of deficits considered, yielding a score from 0 to 1.

#248 · Geriatric Syndromes

What frailty index score is generally considered indicative of frailty?

Answer

A score above approximately 0.25.

#249 · Geriatric Syndromes

What is the Clinical Frailty Scale and who developed the underlying concept?

Answer

A 9-point pictorial and descriptive scale from very fit to terminally ill, developed by Rockwood and colleagues, requiring no special testing.

#250 · Geriatric Syndromes

Why does preoperative frailty assessment matter for elective surgery?

Answer

Frailty independently predicts postoperative delirium, prolonged length of stay, discharge to a facility, and mortality, regardless of chronological age.

#251 · Geriatric Syndromes

What is sarcopenia?

Answer

The age-related loss of skeletal muscle mass, strength, and physical performance.

#252 · Geriatric Syndromes

What is the primary case-finding step in the EWGSOP2 sarcopenia diagnostic algorithm?

Answer

Low muscle strength, most commonly measured by grip strength or the chair stand test.

#253 · Geriatric Syndromes

What confirms a diagnosis of sarcopenia after a positive strength screen?

Answer

Documentation of low muscle quantity or quality by DEXA, bioimpedance analysis, or CT or MRI muscle cross-sectional area.

#254 · Geriatric Syndromes

What additional finding classifies sarcopenia as severe?

Answer

Demonstration of low physical performance, such as slow gait speed, a low SPPB score, or a slow Timed Up and Go.

#255 · Geriatric Syndromes

What daily protein intake is generally recommended for a healthy older adult to preserve muscle mass?

Answer

1.0 to 1.2 grams per kilogram of body weight per day, higher during acute or chronic illness.

#256 · Geriatric Syndromes

What is the intervention with the strongest evidence base for improving strength and function in sarcopenia?

Answer

Resistance exercise training combined with adequate protein intake.

#257 · Geriatric Syndromes

What is the current recommendation regarding pharmacologic treatment of sarcopenia?

Answer

Pharmacologic treatments remain investigational and are not currently recommended as first-line therapy.

#258 · Geriatric Syndromes

What is the classic triad of normal pressure hydrocephalus?

Answer

Gait apraxia, urinary incontinence, and cognitive impairment, with gait disturbance typically the earliest feature.

#259 · Geriatric Syndromes

What gait pattern is characteristic of parkinsonism?

Answer

A shuffling, short-stepped gait with reduced arm swing and difficulty initiating movement.

#260 · Geriatric Syndromes

What gait pattern suggests a cerebellar or sensory ataxic process?

Answer

A wide-based, unsteady gait with variable step length.

#261 · Geriatric Syndromes

What is an antalgic gait and what does it typically suggest?

Answer

A gait with shortened stance phase on the painful limb, suggesting a musculoskeletal source such as hip or knee arthritis.

#262 · Geriatric Syndromes

What gait pattern is expected after a hemispheric stroke with residual leg weakness?

Answer

A hemiplegic or circumductive gait with the affected leg swinging outward.

#263 · Geriatric Syndromes

Why is dual-task gait assessment (walking while performing a cognitive task) clinically useful?

Answer

Impaired dual-task gait performance independently predicts both future falls and incident cognitive decline.

#264 · Geriatric Syndromes

Why is gait speed sometimes called the sixth vital sign in geriatric medicine?

Answer

It is a simple, reproducible measure that independently predicts survival, hospitalization, disability, and institutionalization.

#265 · Geriatric Syndromes

Below what usual gait speed is risk of adverse outcomes considered substantially higher?

Answer

Below 0.6 meters per second.

#266 · Geriatric Syndromes

How is a fragility fracture defined?

Answer

A fracture occurring from a fall from standing height or less.

#267 · Geriatric Syndromes

What should be evaluated after any older adult sustains a fragility fracture?

Answer

Underlying osteoporosis, including bone mineral density testing by DEXA at the hip and spine if not already performed.

#268 · Geriatric Syndromes

Name three secondary causes of osteoporosis to consider when bone loss is disproportionate to age.

Answer

Hyperparathyroidism, hyperthyroidism, vitamin D deficiency, and hypogonadism.

#269 · Geriatric Syndromes

What does the FRAX tool estimate?

Answer

The 10-year probability of hip fracture and major osteoporotic fracture, using clinical risk factors with or without bone mineral density.

#270 · Geriatric Syndromes

Why should pharmacologic osteoporosis therapy be started promptly after hip fracture?

Answer

Treatment delay is associated with excess subsequent fracture risk, particularly in the year after the index fracture.

#271 · Geriatric Syndromes

Which anabolic agents may be preferred over standard antiresorptive therapy in very high fracture risk patients?

Answer

Teriparatide or romosozumab.

#272 · Geriatric Syndromes

What is the essential nonpharmacologic foundation of osteoporosis management alongside medication?

Answer

Calcium and vitamin D repletion, weight-bearing exercise as tolerated, and fall prevention interventions.

#273 · Geriatric Syndromes

What is deconditioning?

Answer

The physiologic decline that follows a period of reduced physical activity, to which older adults are disproportionately vulnerable.

#274 · Geriatric Syndromes

What is hospitalization-associated disability?

Answer

New dependence in basic activities of daily living at discharge that persists at one year, often out of proportion to the acute illness severity.

#275 · Geriatric Syndromes

How does bed rest affect upper versus lower extremity muscle strength?

Answer

Lower extremity strength declines disproportionately compared with upper extremity strength within days of bed rest.

#276 · Geriatric Syndromes

What validated scale assesses pressure injury risk and what domains does it score?

Answer

The Braden Scale, scoring sensory perception, moisture, activity, mobility, nutrition, and friction and shear.

#277 · Geriatric Syndromes

Over which anatomic sites do pressure injuries most commonly develop?

Answer

Bony prominences such as the sacrum, heels, and greater trochanters.

#278 · Geriatric Syndromes

What repositioning interval is generally recommended to prevent pressure injuries in a bed-bound patient?

Answer

Approximately every two hours.

#279 · Geriatric Syndromes

What is a contracture and how is it best prevented?

Answer

Shortening of periarticular soft tissue from a joint held in a fixed position for a prolonged period; prevented with early range-of-motion exercises and appropriate positioning.

#280 · Geriatric Syndromes

What mechanism increases venous thromboembolism risk during immobility?

Answer

Venous stasis from reduced movement of the lower extremities.

#281 · Geriatric Syndromes

When should mechanical VTE prophylaxis such as intermittent pneumatic compression be used instead of pharmacologic prophylaxis?

Answer

When pharmacologic prophylaxis is contraindicated by active bleeding or high bleeding risk.

#282 · Geriatric Syndromes

What single intervention most effectively reduces VTE, deconditioning, delirium, and pressure injury simultaneously?

Answer

Early mobilization after surgery or acute illness, whenever the patient's clinical status allows.

#283 · Geriatric Syndromes

What organizing framework integrates the evaluation and management of falls, frailty, sarcopenia, gait disorders, and immobility complications?

Answer

Comprehensive geriatric assessment, incorporating functional, cognitive, nutritional, and social domains alongside the medical evaluation.

#284 · Geriatric Syndromes

What are the four CAM features and what combination is required to diagnose delirium?

Answer

Acute onset and fluctuating course, inattention, disorganized thinking, and altered level of consciousness. Diagnosis requires features 1 and 2 plus either feature 3 or feature 4.

#285 · Geriatric Syndromes

What are the three motor subtypes of delirium?

Answer

Hyperactive, hypoactive, and mixed. Mixed is the most common subtype in hospitalized older adults.

#286 · Geriatric Syndromes

Why is hypoactive delirium often underdiagnosed?

Answer

It presents as lethargy and psychomotor slowing without agitation, so it is frequently mistaken for depression or fatigue rather than recognized as delirium.

#287 · Geriatric Syndromes

What is the single most important feature distinguishing delirium from dementia?

Answer

Acute onset with a fluctuating course and altered attention in delirium, versus a gradual, stable decline over months to years in dementia.

#288 · Geriatric Syndromes

Name four predisposing factors for delirium.

Answer

Advanced age, preexisting dementia, visual or hearing impairment, and severe underlying illness or multiple comorbidities.

#289 · Geriatric Syndromes

Name five common precipitating factors for delirium.

Answer

Infection, metabolic derangement, medication effects, dehydration, and uncontrolled pain (also urinary retention, hypoxia, and surgery).

#290 · Geriatric Syndromes

Why can a minor precipitant cause delirium in one patient but not another?

Answer

Delirium arises from the interaction of predisposing vulnerability and precipitating insult; a highly vulnerable patient needs only a small trigger, while a robust patient requires a larger insult.

#291 · Geriatric Syndromes

Which Beers Criteria drug class is most strongly linked to precipitating delirium through central cholinergic blockade?

Answer

Anticholinergic agents, including first generation antihistamines like diphenhydramine and bladder antimuscarinics like oxybutynin.

#292 · Geriatric Syndromes

Why should benzodiazepines generally be avoided as first line treatment for delirium-related agitation?

Answer

They frequently worsen rather than improve confusion, with the important exception of delirium from alcohol or benzodiazepine withdrawal, where they are the treatment of choice.

#293 · Geriatric Syndromes

Why is meperidine specifically avoided in older adults?

Answer

Meperidine itself has mild anticholinergic activity, and its metabolite normeperidine is neuroexcitatory (tremor, myoclonus, seizures), not anticholinergic; normeperidine accumulates with renal impairment and repeated dosing, increasing delirium risk.

#294 · Geriatric Syndromes

Why should pain not be undertreated when managing a delirious patient?

Answer

Uncontrolled pain is itself a potent delirium precipitant, so the goal is the lowest effective dose of a safer analgesic, not withholding analgesia.

#295 · Geriatric Syndromes

What is the Hospital Elder Life Program and what does it target?

Answer

A validated multicomponent non-pharmacologic intervention (reorientation, sleep protection, mobilization, sensory aid use) shown to reduce delirium incidence and duration.

#296 · Geriatric Syndromes

When is pharmacologic treatment of delirium-related agitation indicated?

Answer

Only when agitation poses a direct safety risk and has not responded to non-pharmacologic measures, since no medication shortens overall delirium duration.

#297 · Geriatric Syndromes

Which antipsychotic is traditionally favored for delirium agitation and why?

Answer

Low dose haloperidol, because it carries a lower anticholinergic and sedative burden than many alternatives.

#298 · Geriatric Syndromes

Why is quetiapine sometimes preferred over haloperidol in a delirious patient with Parkinson disease?

Answer

Quetiapine carries a lower risk of worsening extrapyramidal symptoms, which haloperidol can exacerbate.

#299 · Geriatric Syndromes

What boxed warning applies to antipsychotic use and why does it matter in delirium management?

Answer

Increased mortality risk in patients with dementia related psychosis, which is why antipsychotic use should be time limited and reserved for genuine safety indications.

#300 · Geriatric Syndromes

Describe the classic history for stress incontinence.

Answer

Small volume leakage triggered by activities that raise intra-abdominal pressure, such as coughing, sneezing, laughing, or exertion, without urgency.

#301 · Geriatric Syndromes

Describe the classic history for urge incontinence.

Answer

A sudden, strong urge to void followed by leakage before reaching the bathroom, often with frequency and nocturia, caused by involuntary detrusor contractions.

#302 · Geriatric Syndromes

Describe the classic history for overflow incontinence.

Answer

Frequent dribbling, a weak stream, hesitancy, and a sense of incomplete emptying, caused by detrusor underactivity or bladder outlet obstruction.

#303 · Geriatric Syndromes

What is functional incontinence and how does it differ from other types?

Answer

Leakage due to immobility or cognitive impairment preventing timely toileting, despite a normal lower urinary tract; managed with scheduled toileting rather than urinary medications.

#304 · Geriatric Syndromes

What single test best distinguishes overflow incontinence from urge incontinence?

Answer

Post-void residual volume; a significantly elevated residual confirms overflow, while urge incontinence leaves a near-empty bladder.

#305 · Geriatric Syndromes

What is the first line treatment for stress incontinence?

Answer

Pelvic floor muscle training.

#306 · Geriatric Syndromes

What is the first line treatment approach for urge incontinence?

Answer

Bladder training combined with an antimuscarinic or beta-3 agonist medication.

#307 · Geriatric Syndromes

Why should antimuscarinic medications be used cautiously in patients with dementia?

Answer

They are anticholinergic and can worsen cognition, and they should not be started until overflow incontinence has been excluded by post-void residual.

#308 · Geriatric Syndromes

What is the most common cause of new fecal incontinence with loose stool in a frail older adult?

Answer

Fecal impaction with overflow diarrhea around a retained stool mass.

#309 · Geriatric Syndromes

What examination finding confirms fecal impaction and what is the appropriate treatment?

Answer

A hard fecal mass on digital rectal examination; treatment is manual disimpaction, not antidiarrheal medication.

#310 · Geriatric Syndromes

What is the appropriate long term management after resolving a fecal impaction?

Answer

A scheduled bowel regimen with adequate dietary fiber and fluid intake to prevent recurrence.

#311 · Geriatric Syndromes

What is presbycusis and what frequencies are affected first?

Answer

Age related sensorineural hearing loss that begins with reduced sensitivity to high frequency sounds and progresses gradually.

#312 · Geriatric Syndromes

Name four consequences of untreated hearing impairment in older adults beyond communication difficulty.

Answer

Social isolation, depression, falls, and increased risk of incident dementia.

#313 · Geriatric Syndromes

What reversible condition must be excluded by otoscopy before attributing hearing loss to presbycusis?

Answer

Cerumen impaction.

#314 · Geriatric Syndromes

Why should a hospitalized patient's hearing aids always be kept in place if possible?

Answer

Removing hearing aids removes a critical source of environmental orientation and can precipitate or worsen delirium.

#315 · Geriatric Syndromes

Describe the typical symptoms of cataracts.

Answer

Painless, progressive blurring of vision, glare with bright lights or headlights at night, and faded color perception.

#316 · Geriatric Syndromes

When is cataract surgery generally recommended?

Answer

Once the cataract begins to interfere meaningfully with daily activities such as driving or reading.

#317 · Geriatric Syndromes

What is the hallmark symptom of age related macular degeneration and what tool detects it?

Answer

Painless central vision distortion, described as straight lines appearing wavy, detected early with an Amsler grid.

#318 · Geriatric Syndromes

Compare the dry and wet forms of macular degeneration.

Answer

Dry form progresses slowly through drusen accumulation; wet form involves choroidal neovascularization and can cause rapid, severe vision loss requiring urgent treatment.

#319 · Geriatric Syndromes

What is the treatment for wet macular degeneration?

Answer

Urgent ophthalmologic referral for intravitreal anti-vascular endothelial growth factor injections.

#320 · Geriatric Syndromes

What is glaucoma and what part of vision is typically affected first?

Answer

A progressive optic neuropathy, usually associated with elevated intraocular pressure, that causes gradual peripheral vision loss before central vision is affected.

#321 · Geriatric Syndromes

Why can glaucoma progress substantially before a patient notices symptoms?

Answer

Because peripheral vision loss is typically asymptomatic until damage is advanced, making patients frequently unaware until significant, irreversible loss has occurred.

#322 · Geriatric Syndromes

How is open angle glaucoma typically detected?

Answer

Through routine screening with tonometry and optic disc examination rather than patient-reported symptoms.

#323 · Geriatric Syndromes

What are the treatment options for glaucoma?

Answer

Topical medications to lower intraocular pressure, with laser or surgical intervention for cases refractory to medical therapy.

#324 · Geriatric Syndromes

How does uncorrected sensory impairment relate to delirium and functional incontinence?

Answer

It is a predisposing factor for delirium and can contribute to functional incontinence by impairing the ability to navigate safely to a toilet.

#325 · Geriatric Syndromes

What is the required initial workup for any older adult with new incontinence?

Answer

Focused history to distinguish incontinence type, medication review, pelvic or prostate exam, urinalysis to exclude infection, and post-void residual measurement.

#326 · Geriatric Syndromes

Why is incontinence never considered a normal part of aging?

Answer

Because it always has an identifiable, often treatable type and cause, so an evaluation should be performed rather than accepting it as inevitable.

#327 · Geriatric Syndromes

What is the effect of chronic antimuscarinic use on bladder emptying and why does this matter?

Answer

It can impair detrusor contractility over time and produce a superimposed overflow component, so a post-void residual should be reassessed if new emptying symptoms develop.

#328 · Geriatric Syndromes

Name three common causes of fecal incontinence other than impaction.

Answer

Anal sphincter weakness from obstetric trauma or surgery, neurologic conditions affecting rectal sensation such as diabetes or dementia, and severe diarrheal illness overwhelming sphincter control.

#329 · Geriatric Syndromes

Why should a delirium workup continue even after one abnormality is found?

Answer

Multiple contributing precipitants are typically present simultaneously, and stopping at the first finding risks missing others that must also be treated.

#330 · Geriatric Syndromes

How should a chronic benzodiazepine be managed in a patient hospitalized with an unrelated acute illness and delirium?

Answer

Avoid abrupt discontinuation to prevent withdrawal while treating the acute illness and other precipitants, then reassess for a supervised taper once stable.

#331 · Geriatric Syndromes

What environmental factors in the intensive care unit contribute to delirium risk?

Answer

Sleep deprivation from nighttime interruptions, sensory deprivation, physical restraints, and an unfamiliar environment.

#332 · Geriatric Syndromes

Why should physical restraints generally be avoided in delirium management?

Answer

They increase agitation and injury risk rather than providing safety.

#333 · Geriatric Syndromes

What laboratory and physical findings should be checked in every delirium evaluation?

Answer

Complete blood count, basic metabolic panel, urinalysis, oxygenation status, pain assessment, and a full medication reconciliation.

#334 · Geriatric Syndromes

Why is a temporal relationship important when linking a new medication to delirium onset?

Answer

An acute confusional change occurring soon after starting or increasing a medication strongly supports a medication-induced etiology, guiding discontinuation as treatment.

#335 · Geriatric Syndromes

What is the appropriate duration for antipsychotic use once started for delirium-related agitation?

Answer

Time limited, with a defined plan to taper and discontinue once the acute safety indication resolves.

#336 · Geriatric Syndromes

What should be done if a patient with suspected urge incontinence also has risk factors for neuropathy, such as diabetes?

Answer

Obtain a post-void residual to exclude an overflow component before starting an antimuscarinic, since neuropathy can impair detrusor contractility.

#337 · Geriatric Syndromes

What is a reasonable interim strategy for a patient who declines hearing aids due to cost?

Answer

Simple communication strategies such as facing the patient, speaking clearly, reducing background noise, and exploring assistance programs.

#338 · Geriatric Syndromes

Why does cataract surgery not affect coexisting glaucoma?

Answer

They are distinct ocular conditions with separate mechanisms, so glaucoma requires its own ongoing monitoring and treatment even after successful cataract surgery.

#339 · Geriatric Syndromes

What does symptom resolution with an antipsychotic NOT confirm about a delirious patient?

Answer

It does not confirm the underlying precipitant has been fully identified or resolved, since antipsychotics control agitation without treating the cause or shortening delirium duration.

#340 · Geriatric Syndromes

How can uncorrected hearing impairment confound bedside cognitive testing?

Answer

It can mimic inattention or cognitive impairment, so sensory input should be optimized, such as with amplification, before formal cognitive assessment.

#341 · Caring for Elderly Patients

What are the six tasks evaluated by the Katz Index of Independence in Activities of Daily Living?

Answer

Bathing, dressing, toileting, transferring, continence, and feeding.

#342 · Caring for Elderly Patients

What eight tasks does the Lawton Instrumental Activities of Daily Living scale assess?

Answer

Managing finances, medications, transportation, telephone use, shopping, meal preparation, housekeeping, and laundry.

#343 · Caring for Elderly Patients

What Timed Up and Go result indicates increased fall risk?

Answer

A time greater than twelve seconds to rise, walk three meters, turn, return, and sit down.

#344 · Caring for Elderly Patients

What gait speed threshold predicts higher risk of disability, hospitalization, and mortality?

Answer

A usual gait speed below 0.6 to 0.8 meters per second.

#345 · Caring for Elderly Patients

What two components make up the Mini-Cog assessment?

Answer

A three-item recall task combined with a clock-drawing test, completed in under three minutes.

#346 · Caring for Elderly Patients

Which pharmacokinetic parameter is least affected by normal aging?

Answer

Drug absorption, since overall bioavailability of most oral drugs is not substantially altered.

#347 · Caring for Elderly Patients

How does aging affect the volume of distribution for lipophilic drugs like diazepam?

Answer

It increases, due to a higher proportion of body fat, which prolongs the drug's elimination half-life.

#348 · Caring for Elderly Patients

How does aging affect the volume of distribution for hydrophilic drugs like digoxin?

Answer

It decreases, due to reduced total body water and lean mass, which can raise peak serum concentrations after a standard dose.

#349 · Caring for Elderly Patients

Why can serum creatinine appear normal despite reduced glomerular filtration rate in older adults?

Answer

Reduced muscle mass lowers creatinine production, masking the true decline in renal function.

#350 · Caring for Elderly Patients

Which benzodiazepines are preferred in older adults and why?

Answer

Lorazepam, oxazepam, and temazepam, because they undergo direct phase II glucuronidation without needing oxidative metabolism.

#351 · Caring for Elderly Patients

How does low serum albumin affect highly protein-bound drugs like phenytoin?

Answer

It increases the free, active drug fraction, which can cause toxicity even when total drug levels appear normal.

#352 · Caring for Elderly Patients

What defines polypharmacy?

Answer

The concurrent use of five or more medications by a patient.

#353 · Caring for Elderly Patients

Name a medication class the Beers Criteria flag to avoid in most older adults due to delirium and fall risk.

Answer

First-generation antihistamines, such as diphenhydramine and hydroxyzine.

#354 · Caring for Elderly Patients

Why do Beers Criteria flag chronic proton pump inhibitor use beyond eight weeks without a clear indication?

Answer

It increases risk of *Clostridioides difficile* infection, bone loss, and vitamin B12 deficiency.

#355 · Caring for Elderly Patients

Why is glyburide specifically flagged in the Beers Criteria?

Answer

Its long duration of action increases the risk of prolonged, sometimes severe hypoglycemia.

#356 · Caring for Elderly Patients

What does the STOPP tool identify?

Answer

Potentially inappropriate medications in older adults, organized by physiological system, to guide deprescribing.

#357 · Caring for Elderly Patients

What does the START tool identify?

Answer

Evidence-based medications that are potentially underused in older adults, addressing the risk of undertreatment.

#358 · Caring for Elderly Patients

What is deprescribing?

Answer

The planned, supervised process of reducing or stopping medications that may cause harm or no longer provide benefit, based on patient goals and life expectancy.

#359 · Caring for Elderly Patients

Why should benzodiazepines be tapered rather than stopped abruptly during deprescribing?

Answer

Abrupt discontinuation risks a known withdrawal syndrome, including seizures and severe rebound anxiety.

#360 · Caring for Elderly Patients

Which drug classes account for most emergency department visits for adverse drug events in older adults?

Answer

Anticoagulants, oral antidiabetic agents and insulin, and opioid analgesics.

#361 · Caring for Elderly Patients

What is anticholinergic burden and why does it matter?

Answer

The cumulative effect of multiple anticholinergic medications, associated with cognitive impairment, delirium, constipation, and long-term dementia risk.

#362 · Caring for Elderly Patients

Name three medication classes commonly implicated in precipitating delirium in hospitalized older adults.

Answer

Benzodiazepines, opioids, and anticholinergic medications, among others such as corticosteroids and high-dose histamine-2 blockers.

#363 · Caring for Elderly Patients

What threshold of unintentional weight loss warrants systematic evaluation in older adults?

Answer

Loss of five percent or more of body weight over six to twelve months.

#364 · Caring for Elderly Patients

What does the Mini Nutritional Assessment evaluate?

Answer

Dietary intake, weight loss, mobility, psychological stress, and body mass index, specifically validated for older adults.

#365 · Caring for Elderly Patients

What is sarcopenia and how is it assessed?

Answer

Age-related loss of skeletal muscle mass and strength, assessed with grip strength, gait speed, and muscle mass imaging.

#366 · Caring for Elderly Patients

Why are older adults more prone to vitamin B12 deficiency?

Answer

Age-related atrophic gastritis reduces intrinsic factor and gastric acid, impairing B12 absorption.

#367 · Caring for Elderly Patients

What effect does vitamin D deficiency have on parathyroid hormone and bone in older adults?

Answer

It contributes to secondary hyperparathyroidism, bone loss, and muscle weakness.

#368 · Caring for Elderly Patients

What sleep architecture changes are expected with normal aging?

Answer

Decreased slow-wave sleep, increased fragmentation, and earlier sleep onset and wake times.

#369 · Caring for Elderly Patients

What does REM sleep behavior disorder suggest as a possible prodromal marker?

Answer

An evolving synucleinopathy, such as Parkinson disease or Lewy body dementia.

#370 · Caring for Elderly Patients

What is the recommended first-line treatment for insomnia in older adults?

Answer

Cognitive behavioral therapy for insomnia, before pharmacotherapy is considered.

#371 • Caring for Elderly Patients

How should clinicians approach sexual health in older patients?

Answer

Proactively and nonjudgmentally, since sexual health remains relevant throughout the lifespan and many concerns are treatable.

#372 • Caring for Elderly Patients

Why is the high-dose or adjuvanted influenza vaccine preferred over standard dose for adults sixty-five and older?

Answer

It improves immunogenicity given age-related decline in immune response.

#373 · Caring for Elderly Patients

What is the current ACIP recommendation for the recombinant zoster vaccine?

Answer

A two-dose series for all adults fifty and older, regardless of prior herpes zoster vaccine history.

#374 · Caring for Elderly Patients

How often should a Td or Tdap booster be updated in adults?

Answer

Every ten years.

#375 · Caring for Elderly Patients

What factor should most heavily influence cancer screening decisions in older adults?

Answer

Estimated remaining life expectancy and the time-to-benefit of the specific screening test.

#376 · Caring for Elderly Patients

What is the primary output of a comprehensive geriatric assessment?

Answer

A prioritized, patient-centered problem list and coordinated care plan.

#377 · Caring for Elderly Patients

Why can a hospitalized older adult with normal creatinine still accumulate a renally cleared drug?

Answer

Reduced muscle mass masks true renal decline, so estimated creatinine clearance, not serum creatinine alone, should guide dosing.

#378 · Caring for Elderly Patients

What is the first step in a structured deprescribing process?

Answer

Reconciling and reviewing the complete current medication list.

#379 · Caring for Elderly Patients

Why are skeletal muscle relaxants like cyclobenzaprine flagged by the Beers Criteria?

Answer

They carry anticholinergic effects, sedation, and increased fracture risk in older adults.

#380 · Caring for Elderly Patients

Why does chronic NSAID use pose particular risk in older adults with heart failure or chronic kidney disease?

Answer

It causes fluid retention and nephrotoxicity, worsening both conditions.

#381 · Caring for Elderly Patients

What is the Medicare Hospice Benefit prognosis requirement for eligibility?

Answer

A life expectancy of six months or less if the illness runs its normal course, certified by two physicians at election.

#382 · Caring for Elderly Patients

How does hospice differ from general palliative care in terms of prognosis?

Answer

Hospice requires a six-month-or-less prognosis and forgoing curative treatment; palliative care has no prognosis requirement and can accompany curative treatment.

#383 · Caring for Elderly Patients

What is the hospice recertification schedule after the initial 180 days?

Answer

Unlimited 60-day benefit periods, each requiring a face-to-face encounter within 30 days before recertification.

#384 · Caring for Elderly Patients

What FAST stage, plus a comorbid complication, supports hospice eligibility in dementia?

Answer

FAST stage 7C or beyond, combined with a complication such as aspiration pneumonia, recurrent fever, or advanced pressure ulcers.

#385 • Caring for Elderly Patients

What heart failure findings support hospice eligibility?

Answer

NYHA class IV symptoms despite optimal medical therapy, an ejection fraction of 20% or less, and symptomatic arrhythmias or syncope.

#386 • Caring for Elderly Patients

What COPD findings support hospice eligibility?

Answer

Disabling dyspnea at rest poorly responsive to bronchodilators, FEV1 typically below 30% predicted, and recurrent hospitalizations for respiratory failure.

#387 · Caring for Elderly Patients

Name the four levels of hospice care under the Medicare Hospice Benefit.

Answer

Routine home care, continuous home care, general inpatient care, and respite care.

#388 · Caring for Elderly Patients

What is continuous home care used for?

Answer

Brief periods of acute symptom crisis requiring intensive nursing presence at home.

#389 · Caring for Elderly Patients

What is respite care and how long can it last?

Answer

Up to five days of inpatient care specifically to give a family caregiver a planned break.

#390 · Caring for Elderly Patients

What tool assesses pain in patients with advanced dementia who cannot self-report?

Answer

The PAINAD scale, assessing breathing, negative vocalization, facial expression, body language, and consolability.

#391 · Caring for Elderly Patients

Why should morphine generally be avoided or dose-reduced in severe renal impairment?

Answer

Morphine-3-glucuronide, its most abundant metabolite, has no analgesic activity but accumulates with renal impairment and causes neuroexcitatory toxicity such as myoclonus; morphine-6-glucuronide, its active metabolite, instead causes enhanced sedation and respiratory depression.

#392 · Caring for Elderly Patients

Which opioid has no clinically significant active renally cleared metabolites?

Answer

Fentanyl, often preferred in advanced kidney disease.

#393 · Caring for Elderly Patients

Why are codeine and tramadol generally avoided in older adults?

Answer

They have variable, unpredictable CYP2D6-dependent metabolism, and tramadol also raises seizure risk.

#394 · Caring for Elderly Patients

Why should a bowel regimen be started with any opioid from the outset?

Answer

Opioid-induced constipation does not develop tolerance over time, unlike sedation or nausea.

#395 · Caring for Elderly Patients

What is the general opioid starting-dose principle for older adults?

Answer

Start at 25 to 50% lower than the standard adult dose with slower titration, sometimes called start low, go slow.

#396 · Caring for Elderly Patients

What is the principle of double effect?

Answer

An intervention given with intent to relieve suffering at a proportionate dose is ethically distinct from euthanasia, even if death is a foreseen possible consequence.

#397 · Caring for Elderly Patients

What is an advance directive and what two components can it include?

Answer

A legal document completed with capacity that may include a living will (treatment preferences) and a durable power of attorney for health care (naming a proxy).

#398 · Caring for Elderly Patients

What is decision-making capacity and how does it differ from legal competence?

Answer

Capacity is a clinical, decision-specific determination made by a physician; competence is a broader legal determination made by a court.

#399 · Caring for Elderly Patients

Name the four elements of the standard capacity assessment framework.

Answer

Understanding, appreciation, reasoning, and the ability to communicate a clear and consistent choice.

#400 · Caring for Elderly Patients

Does a dementia diagnosis automatically mean a patient lacks capacity?

Answer

No. Capacity is decision-specific, and many patients with mild to moderate dementia retain capacity for many decisions.

#401 · Caring for Elderly Patients

Does refusing recommended treatment automatically indicate incapacity?

Answer

No. A patient with capacity has the right to refuse even life-sustaining treatment if the refusal reflects reasoned application of their own values.

#402 · Caring for Elderly Patients

What is the typical default surrogate hierarchy when no proxy is designated?

Answer

Court-appointed guardian, then spouse, then adult children, then parents, then siblings, though order varies by jurisdiction.

#403 · Caring for Elderly Patients

What standard should a surrogate apply when the patient's own wishes are known?

Answer

Substituted judgment, deciding as the patient would have decided based on their known values and prior statements.

#404 · Caring for Elderly Patients

What standard should a surrogate apply when the patient's wishes are unknown?

Answer

The best interest standard, weighing burdens and benefits from a reasonable person's perspective.

#405 · Caring for Elderly Patients

What is a POLST or MOLST form and how does it differ from an advance directive?

Answer

An actionable medical order signed by a clinician that travels with the patient and can be implemented immediately, unlike an advance directive, which typically requires interpretation.

#406 · Caring for Elderly Patients

Who is the intended population for a POLST versus an advance directive?

Answer

POLST is intended for patients with serious illness or frailty facing near-term decisions; advance directives are appropriate for any adult regardless of current health.

#407 · Caring for Elderly Patients

What proportion of Medicare hospitalizations are followed by a 30-day readmission?

Answer

Nearly one in five, with many considered potentially preventable through better transitional care.

#408 · Caring for Elderly Patients

What follow-up timing is recommended for unstable heart failure patients after discharge?

Answer

Within 48 to 72 hours of discharge, given their high risk of early decompensation.

#409 · Caring for Elderly Patients

What is the signature feature of the Naylor Transitional Care Model?

Answer

An advanced practice nurse follows high-risk older adults from hospitalization through the post-discharge period with home visits and phone availability.

#410 · Caring for Elderly Patients

What is the signature feature of Coleman's Care Transitions Intervention?

Answer

A transition coach empowers the patient and caregiver to self-manage using a personal health record, medication list, and red flag checklist.

#411 · Caring for Elderly Patients

What are the core components of Project RED (Re-Engineered Discharge)?

Answer

A comprehensive discharge plan, medication reconciliation, a discharge summary sent to the outpatient provider, and a follow-up call within two to three days.

#412 · Caring for Elderly Patients

What is teach-back technique used for at discharge?

Answer

Confirming patient or caregiver comprehension by having them restate the plan in their own words.

#413 · Caring for Elderly Patients

How does assisted living differ from a skilled nursing facility?

Answer

Assisted living provides housing and help with daily tasks without 24-hour skilled nursing supervision, and is generally not covered by Medicare.

#414 · Caring for Elderly Patients

What Medicare requirement must be met before a skilled nursing facility stay is covered?

Answer

A qualifying inpatient hospital stay, plus an ongoing skilled care need, with coverage limited to 100 days per benefit period.

#415 · Caring for Elderly Patients

What is the Program of All-Inclusive Care for the Elderly (PACE)?

Answer

A capitated Medicare and Medicaid program providing comprehensive interdisciplinary care to nursing-home-eligible individuals who wish to remain in the community.

#416 · Caring for Elderly Patients

What distinguishes elder abuse neglect from self-neglect?

Answer

Neglect is a caregiver's failure to provide necessary care; self-neglect is the older adult's own failure to meet their basic needs.

#417 · Caring for Elderly Patients

What clinical clue suggests financial exploitation in an older adult?

Answer

Unexplained changes in financial status or a sudden, unexplained change to a will or power of attorney.

#418 · Caring for Elderly Patients

Does a capacitated older adult's consent to remain in an abusive situation exempt a physician from mandatory reporting?

Answer

Generally no. Mandatory reporting laws typically still require the report even when the patient has capacity and declines intervention.

#419 · Caring for Elderly Patients

What does evidence show about tube feeding versus careful hand feeding in advanced dementia?

Answer

Tube feeding does not reduce aspiration risk, does not improve survival, and does not improve comfort compared with careful hand feeding.

#420 · Caring for Elderly Patients

What is a time-limited trial in palliative care decision-making?

Answer

A predefined period and specific clinical markers agreed upon in advance to reassess whether an intervention is achieving the patient's goals.

#421 · Geriatric Psychiatry

What are the core clinical features that distinguish dementia with Lewy bodies from Alzheimer disease?

Answer

Fluctuating cognition, recurrent well-formed visual hallucinations, REM sleep behavior disorder, and spontaneous parkinsonism, often with marked neuroleptic sensitivity to antipsychotics.

#422 · Geriatric Psychiatry

What is the one-year rule used to distinguish dementia with Lewy bodies from Parkinson disease dementia?

Answer

If motor parkinsonism precedes cognitive decline by more than one year, the diagnosis is Parkinson disease dementia rather than dementia with Lewy bodies.

#423 · Geriatric Psychiatry

What early features characterize behavioral variant frontotemporal dementia?

Answer

Early personality change, disinhibition, apathy, loss of empathy, compulsive behaviors, and hyperorality, often with relatively preserved memory early on.

#424 · Geriatric Psychiatry

Why is behavioral variant frontotemporal dementia frequently misdiagnosed initially?

Answer

Its early behavioral changes such as disinhibition and poor judgment can closely resemble a primary psychiatric disorder like depression, mania, or a personality disorder.

#425 · Geriatric Psychiatry

What clinical pattern suggests a vascular contribution to a patient's neurocognitive decline?

Answer

A stepwise decline, early executive dysfunction and gait disturbance, focal neurologic signs, and a clear temporal relationship to documented strokes.

#426 · Geriatric Psychiatry

How does the MoCA compare with the MMSE for detecting mild cognitive impairment?

Answer

The Montreal Cognitive Assessment is more sensitive than the MMSE to mild impairment and executive or frontal-subcortical dysfunction.

#427 · Geriatric Psychiatry

What does the Clinical Dementia Rating scale measure?

Answer

It stages dementia severity from 0 (none) to 3 (severe) based on memory, orientation, judgment, community affairs, home and hobbies, and personal care.

#428 · Geriatric Psychiatry

What is the risk of using antipsychotics in a patient with dementia with Lewy bodies?

Answer

Marked neuroleptic sensitivity, which can cause severe, sometimes irreversible worsening with rigidity, autonomic instability, and further cognitive decline.

#429 · Geriatric Psychiatry

What biomarkers support a diagnosis of Alzheimer disease and guide eligibility for anti-amyloid therapy?

Answer

Amyloid PET imaging, CSF amyloid and tau ratios, and plasma phosphorylated tau; confirmed amyloid pathology is required before starting anti-amyloid monoclonal antibodies.

#430 · Geriatric Psychiatry

What is ARIA and why does it matter with anti-amyloid monoclonal antibodies?

Answer

Amyloid-related imaging abnormality, including edema (ARIA-E) and microhemorrhage (ARIA-H); risk is higher in APOE e4 homozygotes and requires serial MRI monitoring.

#431 · Geriatric Psychiatry

How does late-life depression often present differently from depression in younger adults?

Answer

With atypical, somatic features such as fatigue, pain, and cognitive complaints, and irritability or anhedonia rather than an explicit report of sadness.

#432 · Geriatric Psychiatry

What is vascular depression and how does it respond to treatment?

Answer

A depression subtype linked to cerebrovascular disease with prominent executive dysfunction, psychomotor slowing, and reduced insight, responding less robustly to standard antidepressants.

#433 · Geriatric Psychiatry

What clinical features favor pseudodementia over a primary neurocognitive disorder?

Answer

Abrupt onset, short duration, subjective complaints exceeding objective deficits, frequent I don't know answers, effort-dependent testing, and a personal or family history of mood disorder.

#434 · Geriatric Psychiatry

After treating depression in a patient with pseudodementia, what should be done if cognitive symptoms persist?

Answer

Cognition should be reassessed after full depression remission rather than assuming residual deficits are fully explained by mood, since depression can be prodromal to dementia.

#435 · Geriatric Psychiatry

What is the maximum recommended citalopram dose in patients over 60 and why?

Answer

20 milligrams daily, due to dose-dependent QTc prolongation risk that is heightened in older adults.

#436 · Geriatric Psychiatry

When is electroconvulsive therapy particularly indicated in geriatric depression?

Answer

For severe, treatment-resistant, or psychotic depression, including in patients with significant medical comorbidity; it is often underutilized due to stigma.

#437 · Geriatric Psychiatry

What should be evaluated before diagnosing a primary anxiety disorder in a newly anxious older adult?

Answer

Underlying medical contributors such as thyroid dysfunction, cardiac arrhythmia, pulmonary disease, and medication effects, as well as an emerging neurocognitive disorder.

#438 · Geriatric Psychiatry

Why are benzodiazepines avoided as first-line therapy for anxiety in older adults?

Answer

They carry significant fall risk, cognitive impairment, and dependence potential; SSRIs or SNRIs plus cognitive behavioral therapy are preferred instead.

#439 · Geriatric Psychiatry

How does late-onset psychosis typically differ from early-onset schizophrenia?

Answer

Negative symptoms and formal thought disorder are less prominent, while well-systematized paranoid delusions and hallucinations, often tied to sensory impairment, are common.

#440 · Geriatric Psychiatry

What is Charles Bonnet syndrome?

Answer

Visual hallucinations occurring in the setting of significant vision loss, with preserved insight and no other psychiatric symptoms.

#441 · Geriatric Psychiatry

What must be excluded before diagnosing a primary late-onset psychotic disorder?

Answer

Delirium, an underlying neurocognitive disorder, substance effects, and other medical causes of new psychiatric symptoms.

#442 · Geriatric Psychiatry

What is the DICE approach for managing behavioral and psychological symptoms of dementia?

Answer

Describe, Investigate, Create a plan, and Evaluate, a systematic nonpharmacologic framework for addressing BPSD.

#443 · Geriatric Psychiatry

What is the FDA regulatory status of antipsychotics for dementia-related psychosis?

Answer

All antipsychotics carry a black box warning for increased mortality in this population; brexpiprazole is the only agent with specific approval, for agitation in Alzheimer dementia.

#444 · Geriatric Psychiatry

Which second-generation antipsychotic has the most evidence for treating dementia-related agitation?

Answer

Risperidone, used at the lowest effective dose for the shortest duration when nonpharmacologic measures have failed.

#445 · Geriatric Psychiatry

What monitoring is required when using citalopram for dementia-related agitation?

Answer

QTc interval monitoring, given citalopram's dose-dependent effect on cardiac conduction.

#446 · Geriatric Psychiatry

Do cholinesterase inhibitors and memantine reliably treat behavioral symptoms of dementia?

Answer

No; they have only modest and inconsistent effects on BPSD and are primarily indicated for cognitive symptoms.

#447 · Geriatric Psychiatry

Which demographic group has the highest suicide rate of any age and sex category?

Answer

White men over the age of 75.

#448 · Geriatric Psychiatry

Why is the ratio of completed to attempted suicides so much higher in older adults than younger adults?

Answer

Greater lethality of methods used (especially firearms), greater physical frailty reducing survivability, and social isolation delaying discovery and rescue.

#449 · Geriatric Psychiatry

What behavioral warning signs of imminent suicide risk are specific to older adults?

Answer

Giving away possessions, a sudden calm after a period of depression, statements about being a burden, and neglect of medical regimens as passive self-harm.

#450 · Geriatric Psychiatry

What tool is used to systematically assess suicidal ideation and behavior intensity?

Answer

The Columbia-Suicide Severity Rating Scale.

#451 · Geriatric Psychiatry

Why do older adults experience greater intoxication from alcohol at lower doses than younger adults?

Answer

Decreased lean body mass, reduced hepatic metabolism, and increased blood-brain barrier permeability amplify alcohol's effects at a given dose.

#452 · Geriatric Psychiatry

Which benzodiazepines are preferred for alcohol withdrawal in older adults with hepatic impairment?

Answer

Lorazepam and oxazepam, since they are metabolized by glucuronidation rather than extensive hepatic oxidative metabolism.

#453 · Geriatric Psychiatry

How should chronic benzodiazepine use be discontinued in an older adult?

Answer

With a gradual taper over weeks to months, along with patient education about expected withdrawal symptoms such as rebound anxiety and insomnia.

#454 · Geriatric Psychiatry

What should be reviewed regardless of a patient's age before renewing an opioid prescription?

Answer

The prescription drug monitoring program database, along with screening for opioid use disorder.

#455 · Geriatric Psychiatry

When should naloxone co-prescribing be considered for an older adult on chronic opioid therapy?

Answer

When additional risk factors are present, such as concurrent benzodiazepine use or a history of substance use disorder.

#456 · Geriatric Psychiatry

What is hyperorality and in which condition is it a recognized feature?

Answer

Excessive eating of sweets or attempts to eat non-food items; it is a recognized behavioral feature of behavioral variant frontotemporal dementia.

#457 · Geriatric Psychiatry

How do the semantic and nonfluent agrammatic variants of primary progressive aphasia differ?

Answer

The semantic variant shows fluent speech with loss of word and object meaning, while the nonfluent agrammatic variant shows effortful, halting, grammatically impaired speech.

#458 · Geriatric Psychiatry

What MRI finding pattern is typical of Alzheimer disease versus vascular neurocognitive disorder?

Answer

Alzheimer disease shows hippocampal and medial temporal atrophy, while vascular disease shows white matter hyperintensities and lacunar or strategic infarcts.

#459 · Geriatric Psychiatry

What must always be considered first in an older adult with acute new confusion and hallucinations?

Answer

Delirium and its underlying medical causes must be excluded before attributing symptoms to a primary psychiatric disorder.

#460 · Geriatric Psychiatry

Why should nonpharmacologic interventions always be tried first for behavioral symptoms of dementia?

Answer

Because reversible medical and environmental triggers such as pain, constipation, infection, and unmet needs are common and treatable causes of new agitation.

#461 · Gerontology

What is the Hayflick limit?

Answer

The finite number of divisions, roughly 40 to 60, that a normal somatic cell can complete in culture before entering replicative senescence.

#462 · Gerontology

Why do telomeres shorten with each cell division?

Answer

DNA polymerase cannot fully replicate the extreme end of a linear chromosome, the end-replication problem, so telomeric TTAGGG repeats progressively shorten each division.

#463 · Gerontology

Why is mitochondrial DNA more vulnerable to oxidative damage than nuclear DNA?

Answer

It lacks protective histones, has limited repair capacity, and sits in close proximity to reactive oxygen species generated by the electron transport chain.

#464 · Gerontology

What is the senescence-associated secretory phenotype (SASP)?

Answer

The active secretory state of senescent cells, releasing pro-inflammatory cytokines such as IL-6, IL-8, and TNF-alpha that drive chronic inflammaging.

#465 · Gerontology

Define hormesis.

Answer

A mild, repeated cellular stress that produces an adaptive protective response, improving resilience against later, larger stressors; caloric restriction is the classic example.

#466 · Gerontology

What does mTOR do, and what happens when it is inhibited by rapamycin?

Answer

mTOR is a nutrient-sensing kinase that drives growth and protein synthesis while suppressing autophagy; rapamycin inhibition extends lifespan in model organisms.

#467 · Gerontology

What is the Horvath epigenetic clock?

Answer

A tool that estimates biological age from DNA methylation patterns at defined CpG sites, often predicting mortality risk better than chronological age.

#468 · Gerontology

What is loss of proteostasis?

Answer

Declining efficiency of the ubiquitin-proteasome system and autophagy, leading to accumulation of misfolded and aggregated proteins, implicated in neurodegenerative disease.

#469 · Gerontology

Define inflammaging.

Answer

Chronic, low-grade systemic inflammation seen with aging, reflected by elevated IL-6 and CRP, associated with frailty and increased mortality.

#470 · Gerontology

Which age group is the fastest growing segment of the US older adult population?

Answer

The oldest-old, adults aged 85 and above, projected to more than double between 2022 and 2050.

#471 · Gerontology

What is the old-age dependency ratio?

Answer

The ratio of adults 65 and older to the working-age population aged 20 to 64, used to project strain on Social Security, Medicare, and long-term care workforce capacity.

#472 · Gerontology

What primarily explains the historical life expectancy gap between non-Hispanic Black and non-Hispanic white Americans?

Answer

Structural racism, differential access to care, and higher chronic disease burden, not any inherent biological difference in aging.

#473 · Gerontology

Define social determinants of health.

Answer

Nonmedical conditions such as housing stability, transportation access, food security, and health literacy that shape health outcomes, often more powerfully than clinical care.

#474 · Gerontology

How does loneliness compare to smoking and obesity as a mortality risk factor?

Answer

Social isolation and loneliness are independent mortality risk factors with an effect size comparable to smoking or obesity.

#475 · Gerontology

What causes isolated systolic hypertension with widened pulse pressure in normal aging?

Answer

Arterial stiffening from collagen cross-linking and elastin fragmentation in the vessel wall, increasing pulse wave velocity.

#476 · Gerontology

Why does maximal heart rate decline with age?

Answer

Reduced beta-adrenergic receptor responsiveness, not primarily intrinsic sinus node disease.

#477 · Gerontology

What is the approximate normal GFR decline rate with aging after age 40?

Answer

Approximately 1 mL/min/1.73m² per year in many, though not all, individuals.

#478 · Gerontology

Why does phase I hepatic drug metabolism decline with age while phase II is preserved?

Answer

Declining hepatic blood flow and hepatic mass reduce cytochrome P450-mediated phase I metabolism, while conjugation-based phase II reactions are relatively preserved.

#479 · Gerontology

What cognitive domains typically decline with normal aging, and what is preserved?

Answer

Processing speed and working memory decline, while crystallized intelligence and vocabulary remain relatively preserved.

#480 · Gerontology

Define sarcopenia and which fiber type it preferentially affects.

Answer

Progressive age-related loss of skeletal muscle mass and strength, preferentially affecting type II fast-twitch fibers over type I slow-twitch fibers.

#481 · Gerontology

What drives immunosenescence and its effect on vaccine response?

Answer

Thymic involution reduces naive T cell output and shifts the repertoire toward less diverse memory phenotypes, blunting vaccine responses in older adults.

#482 · Gerontology

Is menopause classified as a disease or a normal aging event?

Answer

A universal, physiologic event of normal female reproductive aging, though it carries downstream implications for bone and cardiovascular health.

#483 · Gerontology

How does age-related body composition change affect dosing of lipophilic drugs?

Answer

Increased fat mass and decreased lean mass and total body water increase the volume of distribution of lipophilic drugs, potentially prolonging their effect.

#484 · Gerontology

Define allostatic load.

Answer

Cumulative physiologic wear from chronic stress exposure across neuroendocrine, cardiovascular, metabolic, and immune systems, higher in those facing chronic socioeconomic adversity.

#485 · Gerontology

What did large randomized trials show about antioxidant vitamin supplementation and lifespan?

Answer

They failed to show lifespan or mortality benefit, and some trials showed increased harm, tempering enthusiasm for the free radical theory as a complete explanation.

#486 · Functional Assessment and Rehabilitation

What six domains does the Katz Index of Independence in Activities of Daily Living assess?

Answer

Bathing, dressing, toileting, transferring, continence, and feeding, each scored as independent or dependent for a total score from 0 to 6.

#487 · Functional Assessment and Rehabilitation

In what order are basic ADLs typically lost as functional decline progresses?

Answer

Bathing is usually lost first, followed by dressing, toileting, transferring, and continence, with feeding lost last, roughly the reverse of the order children acquire these skills.

#488 · Functional Assessment and Rehabilitation

What eight domains does the Lawton Instrumental Activities of Daily Living Scale assess?

Answer

Telephone use, shopping, food preparation, housekeeping, laundry, transportation, medication management, and handling finances.

#489 · Functional Assessment and Rehabilitation

Why is IADL decline often clinically useful for detecting early dementia?

Answer

IADL loss typically precedes basic ADL loss and often predates a formal dementia diagnosis by months to years, making it an early functional marker.

#490 · Functional Assessment and Rehabilitation

Why should collateral history be obtained when assessing functional status in a cognitively impaired patient?

Answer

Patients with cognitive impairment commonly overestimate their own abilities, so a discrepancy between patient and caregiver report can be a clue to anosognosia or early dementia.

#491 · Functional Assessment and Rehabilitation

What distinguishes the primary focus of occupational therapy from physical therapy in geriatric rehabilitation?

Answer

Physical therapy addresses gross motor function such as strength, balance, and gait, while occupational therapy addresses fine motor skills, ADL and IADL performance, cognitive strategies, and home safety.

#492 · Functional Assessment and Rehabilitation

How should a single-point cane be sized and on which side should it be held?

Answer

The top of the cane should align with the wrist crease, producing 20 to 30 degrees of elbow flexion, and it should be held in the hand opposite the affected leg.

#493 · Functional Assessment and Rehabilitation

What advantage does a front-wheeled walker have over a standard non-wheeled walker for a patient with Parkinson disease?

Answer

It allows a more natural gait pattern since it does not need to be fully lifted with each step, unlike a standard walker.

#494 · Functional Assessment and Rehabilitation

Which cognitive domains most consistently correlate with driving safety in older adults?

Answer

Visuospatial processing, divided attention, executive function, and processing speed.

#495 · Functional Assessment and Rehabilitation

What is the gold standard evaluation when there is clinical concern about an older adult's fitness to drive?

Answer

A formal on-road driving evaluation performed by a certified driving rehabilitation specialist, since office-based cognitive testing alone has only moderate predictive value.

#496 · Functional Assessment and Rehabilitation

What minimum therapy intensity is required for admission to an acute inpatient rehabilitation facility?

Answer

At least three hours of therapy daily, five to six days per week, across at least two therapy disciplines, with daily or near-daily physician oversight.

#497 · Functional Assessment and Rehabilitation

What defines eligibility for home health services?

Answer

The patient must be homebound, meaning leaving home requires considerable effort and is infrequent, and needs only intermittent skilled care.

#498 · Functional Assessment and Rehabilitation

How does postoperative delirium after hip fracture surgery affect rehabilitation outcomes?

Answer

It significantly impairs participation in rehabilitation and predicts worse functional outcomes and higher mortality.

#499 · Functional Assessment and Rehabilitation

What are the strongest predictors of returning to a prior living situation after hip fracture?

Answer

Premorbid functional status, cognitive status, social support, and the severity of the acute functional insult itself.

#500 · Functional Assessment and Rehabilitation

What interventions define an orthogeriatric comanagement model for hip fracture patients?

Answer

Early mobilization within 24 to 48 hours after surgery, proactive delirium prevention, adequate pain control avoiding oversedation, and early geriatric consultation.